

DenialDesk — Clearinghouse Requirements & Cost Outline

Prepared 2026-09-26 · Consultation document, no code changes · Refs: R-2.2, R-5.5.2, R-7.9.5, REQUIREMENTS Spayer master

Important: all prices below are **indicative market ranges for budgeting only (■■ VERIFY)**. Clearinghouse pricing is negotiated, rarely published, and varies by volume and by whether we contract as a platform/reseller (one contract for all practices) or each practice contracts directly. Confirm every figure with written quotes before committing.

1. Transactions (the core "what for")

Need	X12	What DenialDesk uses it for	Phase	Typical cost (■■ VERIFY)
Claim submission	837P	Send professional claims to payers; start of the Florida prompt-pay clock	MVP	~\$0.10–\$0.50 per claim, or bundled in a per-provider monthly fee
Front-end acknowledgment	999	Confirms the file was syntactically accepted	MVP	Usually included
Claim acknowledgment	277CA	Payer accepted/rejected each claim; proof of receipt date for deadline tracking	MVP	Usually included
Remittance advice (ERA)	835	Payments, adjustments, CARC/RARC denial codes — feeds the denial queue	MVP	\$0–\$0.25 per ERA, or included; some charge per payer enrollment
Eligibility	270/271	Verify coverage at registration; help confirm regime tag (FL-regulated vs ERISA)	Later	~\$0.05–\$0.30 per inquiry
Claim status	276/277	Automated follow-up on unpaid claims before prompt-pay deadlines	Later	~\$0.05–\$0.30 per inquiry
Prior authorization	278	Authorizations (reduces auth-related denials)	Later	Often per-transaction or premium tier
Attachments	275 / portal	Medical records supporting appeals/claims	Later	Often per-attachment fee

2. Data & services

Need	What for	Typical cost (■■ VERIFY)
Payer list with payer IDs (downloadable/API, refreshed)	Seeds our payer master and registration picker; payer ID is required to route claims	Usually included
Payer enrollment support (EDI, ERA, EFT)	Many payers (Medicare/First Coast, FL Medicaid, some Blues) require per-practice enrollment before claims/ERAs flow	Often included; some charge ~\$10–\$50 per payer enrollment
Claim scrubbing / edits	Catch errors before submission, fewer rejections	Included or premium add-on
Paper claim/drop-to-paper for non-EDI payers	Payers that don't accept electronic claims	~\$0.50–\$1.00+ per paper claim
API (REST/SFTP) + sandbox with test payers	Our integration; sandbox needed for CI with synthetic data only	Sandbox usually free; API access may carry platform fee
Reporting / rejection detail	Troubleshoot rejections, SLA tracking	Usually included

3. Contract, compliance & security (non-negotiable for us)

Requirement	Why	Ref
Subcontractor BAA	We are a Business Associate; clearinghouse handles PHI on our behalf	R-2.2, R-5.5.2
U.S.-only data processing/storage in writing	Fla. Stat. § 408.051(3); no offshore processing or support access to PHI	CLAUDE.md #3
SOC 2 Type II report (and/or HITRUST)	Vendor risk management for our own SOC 2	SOC 2 CC9.2
TLS 1.2+, encryption at rest, breach-notice terms	Encryption and incident obligations	CLAUDE.md #6
Uptime SLA, outage communication, data export on exit	Continuity; lesson from 2024 Change Healthcare outage	R-7.9.5
Backup clearinghouse (or dual routing)	Ability to switch if primary goes down	R-7.9.5
Platform/reseller (multi-tenant) agreement	One integration serving many practices, each with own NPI/TIN and enrollments	R-2.2

4. Budget scenarios (illustrative, ■■ VERIFY)

Cost type	Typical range	Notes
Setup / onboarding	\$0 – \$5,000 one-time (platform deals can be higher)	Often waived with volume commitment
Per-provider monthly (bundled model)	~\$50 – \$150 per provider / month	Common for practices; may include unlimited claims + ERAs
Per-transaction (pay-as-you-go model)	See section 1	Better for low volume / early pilots
Monthly minimums (platform model)	~\$500 – \$2,500+ / month	Negotiate low minimums during pilot
Backup clearinghouse	Minimum/standby fee	Needed for R-7.9.5

Example: a 5-provider practice sending ~1,500 claims/month at a bundled \$100/provider ≈ **\$500/month**; the same volume pay-as-you-go at \$0.30/claim + ERAs ≈ **\$450–\$600/month**. Eligibility checks at every visit could add a comparable amount.

5. Candidates to request quotes from

Availity, Waystar, Optum (Change Healthcare), Office Ally, Claim.MD, Stedi (API-first). Also check FL Medicaid (AHCA) and First Coast (Medicare) EDI requirements directly. Ask each for: payer list coverage for Florida, BAA, U.S.-only attestation, SOC 2 report, API/sandbox docs, reseller pricing, and SLA.

6. Next steps

1) Owner requests quotes using the checklist above. 2) architect writes an ADR selecting primary + backup clearinghouse. 3) spec-writer drafts the payer-master spec. 4) edi-x12-specialist builds against the chosen sandbox with synthetic data only.